

# Obstetrics n Gynaecology MADE EASY

COMPILED BY DR.DEEVISH N D

## CLINICAL CASES (OBSTETRICS)

CASE 1 – ANEMIA IN PREGNANCY

CASE 2 – PREGNANCY INDUCED HYPERTENSION (PRE-ECLAMPSIA)

CASE 3 – PREVIOUS CAESAREAN SECTION

CASE 4 – Rh NEGATIVE PREGNANCY

CASE 5 – HEART DISEASE IN PREGNANCY – 1

CASE 6 – HEART DISEASE IN PREGNANCY – 2

### 1. CASE OF ANAEMIA IN PREGNANCY

Name – Vasanthamma  
Age – 30 years  
Address – Nelamangala  
Occupation – Housewife  
Religion – Hindu

Husband's Name – Bailanjappa  
Age – 35 years  
Occupation – Coolie  
Income – Rs. 3300/month  
SE Status – Upper Lower class

G3P2L2 comes with 8 months of amenorrhea

**PRESENTING COMPLAINTS** – Easy fatigability since 2 months

#### **HISTORY OF PRESENTING COMPLAINTS:**

- Patient presents with 8 months of amenorrhea with easy fatigability since 2 months. Previously, the patient was able to do her household work, but for the past 2 months, she gets tired even with minimal work. On walking about 50 m, patient complains of fatigability, giddiness, blurring of vision which is relieved on rest.
- No history of increased bleeding during menses prior to pregnancy.
- No history of exertional dyspnea, palpitation, PND, pedal edema or giddiness.
- No history of bleeding or leak PV.
- No history of bleeding PR or malena.
- No history of passing worms in the stools.
- No history of fever with chills and burning micturation.
- No history of cough with expectoration, hemoptysis, evening rise of temperature or contact with a known case of tuberculosis.
- No history of drug intake (anti-malarial drugs or aspirin).
- No history of any yellowish discolouration of skin and sclera.
- Not a known diabetic or hypertensive.

#### **OBSTETRIC HISTORY:**

Married Life – 13 years, Non-consanguinous  
Obstetric index – G3P2L2

| No. | DELIVERY                        | BABY AT BIRTH                                                              | PRESENT AGE | COMMENTS                                                                         |
|-----|---------------------------------|----------------------------------------------------------------------------|-------------|----------------------------------------------------------------------------------|
| G1  | FTND,<br>Government<br>Hospital | Cried soon after<br>birth, Male, 3.2 kg,<br>Breast fed 3 years             | 12 years    | Booked & Immunized(Had<br>3 ANC visits + TT + IFA)Post<br>partum period – normal |
| G2  | FTND,<br>Government<br>Hospital | Baby cried soon<br>after birth, Female,<br>3 kg, Breast fed – 2<br>½ years | 10 years    | Booked & Immunized(Had<br>3 ANC visits + TT + IFA)Post<br>partum period – normal |

LMP – 02/11/2006

EDD – 09/07/2007

### PRESENT PREGNANCY

T1

- No history of nausea, vomiting or weakness.
- No urinary symptoms
- No drug intake
- No history of craving for abnormal food (pica)

T2

- Quickening in 5th month
- 1st ANC visit – 20 weeks, given TT & IFA tablets (consumed)

T3

- Fetal movements present
- No leak or bleed PV
- No h/o pain abdomen

### CONTRACEPTIVE HISTORY:

No history of using any contraceptive methods.

### MENSTRUAL HISTORY:

Age of Menarche – 13 years

Past Cycles – Regular 30 days cycles with flow lasting 5 days, normal quantity, no pain or passing of clots.

LMP – 02/11/2006

### FAMILY HISTORY:

No history of congenital anomalies or twinning, DM, HTN

### PAST HISTORY:

No history of Tuberculosis, Epilepsy, Asthma  
No history suggestive of any cardiac ailments.  
No history of previous surgeries, blood transfusions.

**PERSONAL HISTORY:**

Diet – Mixed  
Appetite – Good  
Sleep – Sound  
Bowel & Bladder – Regular  
Habits – Nil

**DIET HISTORY:**

Consumes – 2100 kcal/day  
Required – 2400 kcal/day  
Deficit – 300 kcal/day

**GENERAL PHYSICAL EXAMINATION:**

Here is a pregnant lady 30 year old, moderately built and nourished, conscious, alert & cooperative.

Pulse – 84/min, regular, good volume  
BP – 110/68 mm of Hg  
RR – 14/min, regular  
Temperature – Patient is afebrile

Pallor – Present  
Icterus – Absent  
Cyanosis – Absent  
Clubbing – Absent  
Edema – Absent  
Lymphadenopathy – Absent

Thyroid – Normal  
Breasts – Normal  
Spine – Normal

Height – 146 cm  
Weight – 56 kg  
BMI – 26.27

**SYSTEMIC EXAMINATION:**

CVS – S1 S2 heard, No murmurs.  
RS – NVBS heard, no basal crepts.  
CNS – NAD.  
PA – Normal bowel sounds heard

**OBSTETRIC EXAMINATION:**

**INSPECTION:**

- Abdomen is uniformly distended, globular in shape
- Umbilicus everted, hernial orifices normal

- Flanks do not appear to be full
- Stria gravidarum and linea nigra present
- No scars over the abdomen

#### **PALPATION:**

- Abdominal circumference – 76 cm
- Symphysis-fundal height – 28 cm (corresponds to 32 weeks)
- FUNDAL GRIP – Soft, broad & non-ballotable, suggestive of breech
- Lateral Grip – Knob like structures on the right side suggestive of limb buds  
Uniform resistance on the left side suggestive of spine
- 1ST PELVIC GRIP – Smooth, hard, ballotable mass suggestive of head
- 2ND PELVIC GRIP – Fingers converge, head not engaged.
- Uterus is relaxed
- Fetal age =  $28 \times 8/7 = 32$  weeks
- Fetal weight =  $(28-12) \times 155 = 2480$  gm

#### **AUSCULTATION:**

- Fetal Heart sounds heard along the left spino-umbilical line
- 142/min, regular, rhythmic

#### **DIAGNOSIS:**

***30 year old G3P2L2A0 with 32 weeks of gestation, moderate anemia probably due to iron deficiency, not in labour with no clinical signs of failure.***

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### **2. CASE OF PREGNANCY INDUCED HYPERTENSION (PRE-ECLAMPSIA)**

Name – Narayanamma  
Age – 20 years  
Occupation – House wife  
Address – Dairy Circle  
Religion – Hindu  
Date of Admission – 10/07/07

Husband's Name – Chandrababu  
Age – 25 years  
Occupation – Driver  
Income – Rs.1700/per/month  
SE Status – Upper Middle Class  
Date of examination – 12/07/07

G2P0A1 comes with 8 months of amenorrhea.

**PRESENTING COMPLAINTS:** Generalized edema – since 10 days.

#### **HISTORY OF PRESENTING COMPLAINTS:**

- Patient is a gravida 2 para0 presents with generalized edema since 10 days, insidious in onset, initially noticed in the lower limbs which have gradually progressed to involve the upper limbs and face. It is present throughout the day (no diurnal variation), not relieved by overnight rest nor by limb elevation in the morning.
- No history of headache, blurring of vision or syncopal attacks
- No history of reduced urine output, hematuria.
- No history of chest pain, palpitations or breathlessness on exertion or history suggestive of cardiac failure.
- No history of epigastric pain, nausea, vomiting.
- No history of DM or HTN.
- No history of jaundice, ascities before 20 weeks of gestation.

## **OBSTETRIC HISTORY:**

Married Life – 2 years (non – consanguinous marriage)

Obstetric index – G2P0A1

LMP – 03/11/06

EDD – 10/08/07

## **PREVIOUS PREGNANCY**

G1:

- Painless spontaneous abortion at 6th month following bleeding PV. Patient had gone for 4 ANC visits, 2 scans, booked and immunized.
- No history of excessive vomiting. *(Rule out H. mole)*
- No history of HTN during pregnancy.

## **PRESENT PREGNANCY**

T1

- Morning sickness for 2 months – present.
- Increased frequency of micturation – present.
- No history of easy fatiguability.
- No history of discharge or bleed PV.
- No history of drug intake or radiation exposure.
- No history of Pica.

T2

- Quickening at 5th month.
- No history of headache, blurred vision or sudden increase in weight.
- Booked and Immunized – 3 ANC visits, 2 TT, 100 IFA, Scan done at 20th week.

T3

- Fetal movements present.
- No history of bleeding or discharge PV.
- No history of pain abdomen.
- Generalized edema – present.
- Last abortion – 1 year back.

## **MENSTRUAL HISTORY:**

Age of Menarche – 16 years

Past Cycles – Regular, 30 day cycle, 4 days flow, no pain or passage of clots.

LMP – 03/11/06

No history of any contraceptives used.

**FAMILY HISTORY:** No history of DM, HTN, asthma, twinning in family. No history of PIH in mother or sister.

## **PAST HISTORTY:**

Medical – No history suggestive of DM/HTN.No history of TB, epilepsy or asthma.

Surgical – No history of blood transfusions or any previous surgical procedures.

## **PERSONAL HISTORY:**

Diet – Mixed

Appetite – Good

Sleep – Sound

Bowel & Bladder – Regular

Habits – Nil

## **GENERAL PHYSICAL EXAMINATION:**

Here is a pregnant lady, moderately built and nourished, conscious, alert & cooperative; well oriented to time, place and person.

Pulse – 86/min, regular, good volume

BP – 146/92 mm of Hg

RR – 18/min, regular

Temperature – Patient is afebrile

Pallor – Present

Icterus – Absent

Cyanosis – Absent

Clubbing – Absent

Edema (pedal) – Present, Pitting in nature

Lymphadenopathy – Absent

Thyroid – Normal

Breasts – Normal

Spine – Normal

Gait – Normal

Height – 160 cm

Weight – 70 kg

BMI – 27.3

## **SYSTEMIC EXAMINATION:**

CVS – S1 S2 heard, no murmurs.

RS – NVBS heard, no additional sounds heard.

CNS – Knee jerk – present. Sensory, motor and cranial nerves – normal.

PA – Normal bowel sounds heard

## **OBSTETRIC EXAMINATION:**

### **INSPECTION:**

- Abdomen uniformly distended.
- Flanks not full.
- Umbilicus – everted.
- Striae gravidarum, albicans & linea nigra – present.
- No scars over abdomen, no dilated veins.
- Hernial orifices – normal.

**PALPATION:** (Patient examined in supine position with legs semi flexed).

- Fundal height corresponds to 32 weeks gestation.
- SFH is 28 cm, abdominal circumference – 85 cm.

- Fundal grip – Smooth, broad irregular structure suggestive of breech.
- Lateral Grip – Right – Knob like structures suggestive of limb buds.  
Left – Uniform curved resistance suggestive of spine.
- 1st Pelvic Grip – Smooth, round, hard ballotable mass (not engaged) suggestive of head felt at lower pole

#### **AUSCULTATION:**

- FHS heard along the left spino-umbilical line, mid point.
- Rate – 146/min, regular.

#### **DIAGNOSIS:**

***20 year old G2A1 with 32 weeks gestation, single live fetus with cephalic presentation with head not engaged and not in labour, with mild pre-eclampsia (on treatment) complicating her pregnancy.***

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### **3. CASE OF PREVIOUS LOWER SEGMENT CAESAREAN SECTION**

Name – Anita  
Age – 23 years  
Address – Atmajyothinagar, Kengeri  
Occupation – Maid servant  
Religion – Hindu  
Date of admission – 09/07/2007

Husband's Name – Venkatesh  
Age – 24 years  
Occupation – Painter  
Income – Rs.2600/mnt  
SE Status – Lower Middle Class  
Date of examination – 10/07/2007

G2P1L1 comes with 9 months of amenorrhea for safe confinement

#### **HISTORY OF PRESENTING COMPLAINTS:**

- Patient comes with 9 months amenorrhea with a history of previous LSCS and was admitted for safe confinement. Patient had been here for regular ANC checkup on 27/07/2007 and was asked to get admitted as her EDD as per scan was 10/07/2007.
- Patient complaints of backache since today morning in the lower mid-back, non-radiating and not associated with pain abdomen.
- Patient gives history of white discharge since 1 week, non-foul smelling, not associated with fever or itching.
- No history of leak PV or bleeding PV.
- No history of hematuria.
- No history of any change in bladder habits.
- Fetal movements are well perceived.
- No history of Diabetes mellitus or Hypertension.

#### **OBSTETRIC HISTORY:**

Married Life – 4 years (non – consanguineous marriage)  
Parity index – G2P1L1

LMP – 01/11/06  
EDD – 08/08/07

#### **PREVIOUS PREGNANCY:**

- History of increased vomiting – present.
- History of easy fatigability.
- No history of urinary symptoms.
- No history of drug intake or radiation exposure.
- No history of pica.

T2

- Quickening at 20th week.
- History of generalized edema – present.
- No history of headache or blurring of vision.
- Patient was booked and immunized – 6 ANC checkups, 2 USG scans, 2 TT & 100 IFA.

T3

- Fetal movements present.
- Uneventful.
- Delivered by Lower Segment Caesarean Section probably due to obstructed labour or non-progression of labour.
- Patient was initially put on trial of labour by administering injections, but since labour pains were not adequate, she was posted for emergency LSCS, after infusing 1 unit of blood.
- Outcome was a live male fetus, 3.7 kg at birth, was immunized and exclusively breast fed for 1 year.
- Mother had no fever or wound discharge in the post-op period.
- Sutures were removed on the 7th day but had to stay in the hospital for 16 days as the baby had jaundice.
- Last C-section – 3 years back (April 25th, 2004)

**PRESENT PREGNANCY:** T1, T2 and T3 uneventful. EDD-08/08/07

**CONTRACEPTIVE HISTORY:**

No history of using any contraceptive methods.

**MENSTRUAL HISTORY:**

Age of Menarche – 12 years

Past Cycles – Regular, 50-70 day cycle, 8-9 days flow, no pain or passage of clots.

LMP – 01/11/06

No history of any contraceptives used.

**FAMILY HISTORY:** No history of DM, HTN.

**PAST HISTORY:**

Medical – No history suggestive of DM/HTN. No history of TB, epilepsy or asthma.

Surgical – No history of blood transfusions or any previous surgical procedures.

**PERSONAL HISTORY:**

Diet – Mixed

Appetite – Good

Sleep – Sound

Bowel & Bladder – Regular

Habits – Nil

## **GENERAL PHYSICAL EXAMINATION:**

Mother is a young lady, moderately built and nourished, conscious, alert & cooperative; well oriented to time, place and person.

Pulse – 78/min, regular, good volume  
BP – 116/82 mm of Hg  
RR – 18/min, regular  
Temperature – Patient is afebrile

Pallor – Present  
Icterus – Absent  
Cyanosis – Absent  
Clubbing – Absent  
Edema – Absent  
Lymphadenopathy – Absent

Thyroid – Normal  
Breasts – Normal  
Spine – Normal  
Gait – Normal

Height – 158 cm  
Weight – 51 kg

## **SYSTEMIC EXAMINATION:**

CVS – S1 S2 heard, No murmurs.

RS – NVBS heard, no basal crepts.

CNS – NAD.

PA – NAD

## **OBSTETRIC EXAMINATION:**

### **INSPECTION:**

- Distended and flanks are full.
- Umbilicus – normal.
- Striae gravidarum, albicans & linea nigra – present.
- No dilated veins.
- Hernial orifices – normal.
- A vertical right paramedian incision, 14 cm long is seen in the infra-umbilical region, healed by primary intention – no hypertrophy or keiloid formation, no supra-pubic bulge.

**PALPATION:** (Patient examined in supine position with legs semi flexed).

- Fundal height corresponds to 32 weeks with flanks full – corresponding to 40 weeks of gestation.
- SFH is 32cm.
- Fundal grip – Broad, soft irregular structure suggestive of breech.
- Lateral Grip – Right – Knob like structures suggestive of Limb buds.  
Left – Uniform curved resistance suggestive of spine.
- 1st Pelvic Grip – Smooth, hard ballotable mass.
- 2nd Pelvic Grip – Fingers diverge.
- Abdominal girth – 95 cm.

- Weight of the fetus (Johnson's formula) = 3260 gm.
- Age of fetus (Mc Donald's formula) = 40 weeks.
- No scar tenderness.
- No supra-pubic bulge felt.

#### **AUSCULTATION:**

- FHS heard along the left spinoumbilical line, mid point.
- Rate – 140/min, regular.

#### **DIAGNOSIS:**

***23 year old G2P1L1 with full term single intrauterine pregnancy with previous LSCS with longitudinal lie with cephalic presentation not in labour.***

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### **4. CASE OF Rh NEGATIVE PREGNANCY**

Name – Savita

Age – 24 years

Occupation – House wife

Address – Chamrajpet

SE Status – Lower Middle Class

Date of Admission – 07/07/07

Husband's Name – Satishchandra

Age – 28 years

Occupation – Clerk

Income – Rs. 1000/person/month

Date of examination – 11/07/07

G2P1Lo comes with 7 months of amenorrhea for safe confinement.

#### **HISTORY OF PRESENTING COMPLAINTS:**

- Patient comes with 7 months amenorrhea for safe confinement. Patient had been here for regular ANC checkup on 5th July and was advised to get admitted telling her that her blood group does not match with that of her baby (told to her by a private practitioner).
- No history of generalized weakness and giddiness
- No history of headache, blurred vision or decreased micturition
- No history of edema and pruritis.
- No other systemic complaints.

#### **OBSTETRIC HISTORY:**

Married Life – 4 years (non – consanguineous marriage)

Obstetric index – G2P1L0A0D1

LMP – 04/12/06

EDD – 11/08/07

#### **PREVIOUS PREGNANCY:**

- FTD at home, cried soon after birth, weight not measured.
- Booked & Immunized, 5 ANC visits, 2 TT & 100 IFA.
- The baby died 2 days after birth due to unknown reasons.

#### **PRESENT PREGNANCY**

- Morning sickness for 2 months.
- No history of Urinary symptoms.
- No history of Drug intake.
- No history of Pica.

T2

- Quickening at 20th week.
- No history of headache, blurred vision.
- 2 ANC visits, 2 TT, 100 IFA, 2 scans.

T3

- Fetal movements present.
- No bleeding/leak PV.
- In this pregnancy, she was evaluated & her blood group turned out to be B –ve while that of the fetus was O +ve
- No Anti – D injection given.
- No history of abortion, LSCS or IUFD or invasive fetal procedure.
- Previous baby blood group not known.
- Last delivery – 2 years back.

#### **CONTRACEPTIVE HISTORY:**

No history of using any contraceptive methods.

#### **MENSTRUAL HISTORY:**

Age of Menarche – 15 years

Past Cycles – Regular, 30 day cycle, 4 days flow, no pain or passage of clots.

LMP – 04/12/06

**FAMILY HISTORY:** No history of DM, HTN.

#### **PAST HISTORY:**

Medical – No history suggestive of DM/HTN. No history of TB, epilepsy or asthma.

Surgical – No history of blood transfusions or any previous surgical procedures.

#### **PERSONAL HISTORY:**

Diet – Mixed

Appetite – Good

Sleep – Sound

Bowel & Bladder – Regular

Habits – Nil

#### **GENERAL PHYSICAL EXAMINATION:**

Mother is a 24 year old lady, moderately built and nourished, conscious, alert & cooperative.

Pulse – 82/min, regular, good volume

BP – 120/50 mm of Hg

RR – 18/min, regular

Temperature – Afebrile

Pallor – Absent  
Icterus – Absent  
Cyanosis – Absent  
Clubbing – Absent  
Edema – Absent  
Lymphadenopathy – Absent

Thyroid – Normal  
Breasts – Normal  
Spine – Normal  
Gait – Normal

Height – 156 cm  
Weight – 60 kg

#### **SYSTEMIC EXAMINATION:**

CVS – S1 S2 heard, No murmurs.

RS – NVBS heard, no basal crepts.

CNS – NAD.

PA – NAD

#### **OBSTETRIC EXAMINATION:**

##### **INSPECTION:**

- Abdomen uniformly distended.
- Flanks not full.
- Umbilicus – normal.
- Striae gravidarum, albicans & linea nigra – present.
- No scars over abdomen, no dilated veins.
- Hernial orifices – normal.

**PALPATION:** (Patient examined in supine position with legs semi flexed).

- Fundal height corresponds to 28 weeks gestation.
- SFH is 25 cm.
- Fundal grip – Smooth, broad irregular structure suggestive of breech.
- Lateral Grip – Right – Knob like structures suggestive of Limb buds.  
Left – Uniform curved resistance suggestive of spine.
- 1st Pelvic Grip – Smooth, round, hard ballot able mass (not engaged) suggestive of Head felt at lower pole.

##### **AUSCULTATION:**

- FHS heard along the left spinoumbilical line, mid point.
- Rate – 140/min, regular.

#### **DIAGNOSIS:**

***22 year old G2P1Lo with 7 months amenorrhea, single live fetus, not in labour with Rh –ve pregnancy.***

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Name – Chandrakala  
Age – 32 years  
Address – Chikaballapur  
Occupation – Housewife  
Religion – Hindu  
Date admission – 12/07/2007

Husband's Name – Manjunath  
Age – 35 years  
Occupation – Cloth merchant  
Income–Rs.2000/month  
SE Status – Upper Middle  
Date of examination– 12/07/2007

G3P1L1A1 comes with 9 months of amenorrhea for safe confinement of delivery.

**HISTORY OF PRESENTING COMPLAINTS:**

- Patient comes with 9 months amenorrhea for safe confinement with a history of cardiac surgery.
- No history of breathlessness on exertion, palpitations, chest pain, PND, orthopnea, edema of feet.
- No history of any congenital heart disease.
- No history suggestive of CCF, infective endocarditis in the past or present pregnancy.

**OBSTETRIC HISTORY:**

Married Life – 16 years (non – consanguineous marriage)  
Obstetric index – G3P1L1A1

LMP – 15/10/06  
EDD – 22/07/07

**PREVIOUS PREGNANCY:**

G1 – FTND, Government Hospital, Now 11 years, Cried soon after birth, Weighed 3 kg, Post partum period normal, Booked and immunized, 3 ANC visits, 2TT & 100 IFA received.

G2 – Aborted at 1½ months gestation (MTP) 6 years ago.

**PRESENT PREGNANCY:**

T1

- History of nausea and vomiting.
- No history of urinary symptoms.
- No history of drug intake or radiation exposure.
- No history of pica.

T2

- Quickening at 18th week.
- No history of headache or blurring of vision or edema.
- Patient was booked and immunized – 4 ANC checkups, 2 TT & 100 IFA.

T3

- Increased frequency of micturition – present.
- Fetal movements present.
- Uneventful.

**CONTRACEPTIVE HISTORY:**

No history of using any contraceptive methods.

**MENSTRUAL HISTORY:**

Age of Menarche – 15 years

Past Cycles – Regular, 30 day cycle, 3 days flow, no pain or passage of clots.

LMP – 15/10/06

**FAMILY HISTORY:** No history of DM, HTN. No history of any congenital heart disease among relatives.

**PAST HISTORY:**

- Patient underwent a cardiac surgery 2 years back when she developed sudden onset of breathlessness though she was on medical treatment for some cardiac ailment for 5 years. Her previous reports revealed that she was diagnosed to have RSOV with VSD. She underwent the operation in a government hospital in Putbarti.
- No history of fleeting joint pains or fever in the childhood and patient not on penidure prophylaxis.
- No history of any post-op complications.
- No history suggestive of DM or HTN.
- No history of TB, epilepsy or asthma.

**PERSONAL HISTORY:**

Diet – Mixed

Appetite – Good

Sleep – Sound

Bowel & Bladder – Regular

Habits – Nil

**GENERAL PHYSICAL EXAMINATION:**

Here is a pregnant lady, moderately built and nourished, conscious, alert & cooperative; well oriented to time, place and person.

Pulse – 90/min, regular, good volume, normal character, all PP felt. JVP – normal

BP – 130/70 mm of Hg

RR – 18/min, regular, TA

Temperature – Afebrile

Pallor – Absent

Icterus – Absent

Cyanosis – Absent

Clubbing – Absent

Edema – Absent

Lymphadenopathy – Absent

Thyroid – Normal

Breasts – Normal

Spine – Normal

Gait – Normal

Height – 160 cm

Weight – 60 kg

## **SYSTEMIC EXAMINATION:**

### **CARDIO-VASCULAR SYSTEM:**

#### **INSPECTION**

- No precordial bulge.
- Apical impulse – left 4th inter-costal space, 2 cm lateral to Mid-cavicular line.
- No other abnormal pulsations.
- A linear scar seen over the mid-sternum 15 cm × 2 cm.
- No dilated veins over the chest wall.

#### **PALPATION**

- Inspectory findings were confirmed.
- Apex beat – left 4th inter-costal space, 2 cm lateral to Mid-cavicular line.
- No parasternal heave.
- No thrill felt.
- No abnormal pulsations.

#### **AUSCULTATION**

##### **CVS**

|                       |                         |
|-----------------------|-------------------------|
| <b>Aortic area</b>    | S1S2 heard, no murmurs. |
| <b>Pulmonary area</b> |                         |
| <b>Mitral area</b>    |                         |
| <b>Tricuspid area</b> |                         |

RS – NVBS heard, no basal crepts.

CNS – NAD.

PA – NAD

## **OBSTETRIC EXAMINATION:**

#### **INSPECTION:**

- Abdomen is distended, flanks are full.
- Umbilicus – normal.
- Striae gravidarum, albicans & linea nigra – present.
- No dilated veins or scars or sinuses.
- Hernial orifices – normal.

**PALPATION:** (Patient examined in supine position with legs semi flexed).

- Fundal height corresponds to 32 weeks with flanks full – corresponding to 40 weeks of gestation.
- Shelving Sign – positive.
- Symphysis – fundal height is 30 cm.
- Fundal grip – Broad, soft, non-ballotable, relatively large irregular structure suggestive of breech.
- Lateral Grip – Right – Knob like structures suggestive of Limb buds.  
Left – Uniform curved resistance suggestive of spine.
- 1st Pelvic Grip – Smooth, hard ballotable mass relatively small felt suggestive of head.

- Abdominal girth – 104 cm.
- Weight of fetus (Johnson's formula) – 2800 gm.
- Age of fetus (Mc Donald's formula) – 40 weeks.

#### **AUSCULTATION:**

- FHS heard along the left spinoumbilical line, mid point.
- Rate – 140/min, regular.

#### **DIAGNOSIS:**

***32 year old G3P1L1A1 with full term pregnancy with cephalic presentation, not in labour with a previous history of cardiac surgery.***

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### **6. CASE OF HEART DISEASE IN PREGNANCY – 2**

Name – Farida Taj

Age – 25 years

Address – Chikaballapur  
seller

Occupation – Worker in Agarbatti factory

Religion – Hindu

Date of admission – 08/11/2007  
21/11/2007

Husband's Name – Rehman

Age – 30 years

Occupation – Plastic Items

Income–Rs.3000/month

SE Status – Upper Middle Class

Date of examination –

Primigravida comes with 9 months of amenorrhea

#### **PRESENTING COMPLAINTS:**

- Pain abdomen – 13 days.
- Swelling of both lower limbs – 13 days.
- Chest pain and breathlessness – 8 days.

#### **HISTORY OF PRESENTING COMPLAINTS:**

- Patient gives history of pain abdomen for the past 13 days, over the lower part of the abdomen, moderate intensity, intermittent in nature, each episode lasting about 2 hours and approximately 2-3 episodes per day, relived on medication.
- Patient also complaints of swelling of both the lower limbs since 13 days, insidious in onset, initially present over the feet and has gradually progressed to the knee, present throughout the day, increases on walking and relived on taking rest. No diurnal variation. No history of distention of abdomen or puffiness of face.
- Patient also gives a history of chest pain since last 8 days, sudden in onset, over the retrosternal region, progressive, constricting type, non-radiation, moderate severity, aggravated on exertion and relieved on rest. It is associated with breathlessness, insidious in onset, progressive in nature, initially patient was able to do her routine activities but now she gets breathless after walking a few meters. It is relieved on rest.
- History of palpitations present.
- No history of bleeding or discharge per vagina.
- No history of orthopnea, PND.
- No history suggestive of CCF, Infective endocarditis.
- No history of fever.
- No history suggestive of thyroid disease.

- No history of any cardiac disease
- Not a known case of DM or HTN.

### **OBSTETRIC HISTORY:**

Married Life – 1 years (non – consanguineous marriage)

Parity index – primigravida

LMP – 03/03/07

EDD – 10/12/07

### **PRESENT PREGNANCY:**

T1

- History of nausea and vomiting.
- History of urinary symptoms – present.
- No history of drug intake or radiation exposure.
- No history of pica, Booked and Immunized.

T2

- Quickening at 5th month.
- No history of headache or blurring of vision or edema.

T3

- Fetal movements present.
- Developed swelling of both lower limbs, chest pain and breathlessness as mentioned previously.

### **CONTRACEPTIVE HISTORY:**

No history of using any contraceptive methods.

### **MENSTRUAL HISTORY:**

Age of Menarche – 15 years

Past Cycles – Regular, 30 day cycle, 3 days flow, no pain or passage of clots.

LMP – 03/03/07

**FAMILY HISTORY:** No history of DM, HTN. No history of any congenital heart disease among relatives.

### **PAST HISTORTY:**

- No history of fleeting joint pains or fever in the childhood and patient not on penidure prophylaxis.
- No history suggestive of any other congenital heart disease.
- No history of heart surgery.
- No history suggestive of DM or HTN.
- No history of TB, epilepsy or asthma.
- No history of previous hospitalization or treatment for heart ailments.

### **PERSONAL HISTORY:**

Diet – Mixed  
Appetite – Good  
Sleep – Sound  
Bowel & Bladder – Regular  
Habits – Nil

### **GENERAL PHYSICAL EXAMINATION:**

Mother is a young lady, moderately built and nourished, conscious, alert & cooperative; well oriented to time, place and person.

Pulse – 99/min, regular, good volume, normal character, all PP felt. JVP– raised (6 cm).  
BP – 126/90 mm of Hg in left upper limb in supine position.  
RR – 18/min, regular, TA  
Temperature – Patient is afebrile

Pallor – Absent  
Icterus – Absent  
Cyanosis – Absent  
Clubbing – Absent  
Edema – Absent  
Lymphadenopathy – Absent

Thyroid – Normal  
Breasts – Normal  
Spine – Normal  
Gait – Normal

Height – 160 cm  
Weight – 60 kg

### **SYSTEMIC EXAMINATION:**

#### **CARDIO-VASCULAR SYSTEM:**

##### **INSPECTION**

- No precordial bulge.
- Apical impulse – left 4th inter-costal space, 2 cm lateral to Mid-cavicular line.
- No other abnormal pulsations.
- No dilated veins over the chest wall, no scars.

##### **PALPATION**

- Inspectory findings were confirmed.
- Apex beat – left 4th inter-costal space, 2 cm lateral to Mid-cavicular line.
- Parasternal heave – present.
- No thrill felt.
- No abnormal pulsations.

##### **AUSCULTATION**

##### **CVS**

|                |                                |
|----------------|--------------------------------|
| Aortic area    | S1 loud, S2 heard , No murmurs |
| Pulmonary area | ESM – present                  |
| Mitral area    | MDM – present                  |
| Tricuspid area | S1S2 heard, No murmurs         |

**RS** – NVBS heard, no basal crepts.

**CNS** – NAD.

**PA** – NAD

## **OBSTETRIC EXAMINATION:**

### **INSPECTION:**

- Abdomen is distended, flanks are full.
- Umbilicus – normal.
- Striae gravidarum, albicans & linea nigra – present.
- No dilated veins or scars or sinuses.
- Hernial orifices – normal.

### **PALPATION:**

- Abdominal circumference – 76 cm
- Symphysio-fundal height – 28 cm (corresponds to 32 weeks)
- FUNDAL GRIP – Soft, broad & non-ballotable, suggestive of Breech
- Lateral Grip – Knob like structures on the right side suggestive of limb buds  
Uniform resistance on the left side suggestive of spine
- 1ST PELVIC GRIP – Smooth, hard, ballotable mass suggestive of head
- 2ND PELVIC GRIP – Fingers converge, head not engaged.
- Uterus is relaxed

### **AUSCULTATION:**

- FHS heard along the left spinoumbilical line, mid point.
- Rate – 140/min, regular.

## **DIAGNOSIS:**

***25 year old primi with full term pregnancy with cephalic presentation not in labour with cardiac disease (valvular lesion), probably RHD, MS in sinus rhythm, not in failure with no evidence of infective endocarditis.***

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**Ashi**

May 2, 2011 at 8:15 pm

super stuff !! very helpful too !!

[Reply](#)

**ramya**

January 10, 2013 at 12:22 am

thanks a lot sir.... itz very helpful....

[Reply](#)

**Hasna**

June 13, 2013 at 11:18 pm

Excellent work..really useful too..

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